Methotrexate: head and neck cancer

National Cancer Control Programme (NCCP) 00893 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.

STEP 2

Taking or receiving treatment

Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself.

STEP 3

Tests and reviews

Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.

STEP 4

Specific practical instructions

Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Sources: NCCP 00893 v1: official regimen. Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending.